PC-03 Pediatric Pulmonary Edema / CHF

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Infant or child after the newly born period with suspected acute heart failure, pulmonary edema, or cardiogenic shock. Congenital anatomy and baseline physiology may change usual findings and oxygen targets.

Field flow

1. Assess	Respiratory distress, perfusion, cardiac history/surgeries, baseline SpO2, home medications, fluid restriction, feeding/urine output, fever, and caregiver baseline.
2. Support	Position of comfort; oxygen and ventilation support for distress. Use gentle positive pressure and contact Medical Control early for complex congenital disease.
3. Baseline target	Do not force normal SpO2 in cyanotic or single-ventricle disease; use documented/caregiver baseline and Medical Control guidance.
4. Monitor	Continuous ECG, SpO2, frequent BP, respiratory reassessment, and EtCO2 when ventilatory support is used.
5. Transport	No routine fluid bolus. Notify early and transport without delaying for a medication trial.

Provider-level actions

Level	Approved actions
EMT	Airway/ventilation support, SpO2 and frequent vital signs, focused cardiac history, and communicate usual saturation/anatomy/medication plan.
AEMT	All EMT care plus indicated IV/IO, ECG/EtCO2, early Medical Control, and cautious reassessed aliquots only if hypovolemia is genuinely suspected.
Paramedic	All prior care plus cardiac/12-lead monitoring without delaying care and Medical-Control-directed therapy tailored to anatomy and physiology.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Do not use routine albuterol, fluid, vasodilator, or vasoactive treatment without an identified indication and Medical Control direction.

• Do not force a normal SpO2 in cyanotic or single-ventricle disease without individualized guidance.

• Use PM-01 for suspected anaphylaxis; use the appropriate cardiac pathway for bradycardia or arrest.

Medication reference

Medication	Approved dose / safeguard
Fentanyl	1 mcg/kg IV/IO; max single dose 50 mcg only with Medical Control after airway and perfusion stabilize.
Vasodilator / vasoactive	Agent, dose, and indication determined by Medical Control.

Linked pathways

AR-05 Pediatric Airway; AR-07 Pediatric Respiratory Distress; PM-01 Pediatric Anaphylaxis; PC-02 Bradycardia / Poor Perfusion; PC-04 Pulseless Arrest.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.